

Anesthesia Record

Patient Demographics

Name	Patient ID	SSN	Gender Identity	Birth Date
Address	Phone	Email		
Reg Status	PCP	Date Last Verified	Next Review Date	

Subscriber Details

				Hospital Account
CVG	Subscriber Name/Sex/Relation	Subscriber DOB	Subscriber Address/Phone	Subscriber Emp/Emp Phone

Guarantor Information

Name:		SSN:	
Address:			
City:	State:	Zip:	Phone:
Employer:			
Address:			
City:	State:	Zip:	Phone:
Guar DOB:			

Anesthesia Summary -

y Current as

Height: 1.6 m (5' 3") (04/25/23)
Weight: 68 kg (150 lb) (04/25/23)
BMI:
NPO Status: 0000
Allergies: PENICILLINS

General Information

Date: 4/25/2023	Case classification: Scheduled	Status: Unposted
Location:	Room: OR 03	Service: Neurosurgery
Patient class: Hospital Ambulatory Surgery		

Procedure Info

Panel 1				
Surgeon	Role		Service	
	Primary		Neurosurgery	
	Assisting		Neurosurgery	
Procedure: L4-L5 LAMINECTOMY ABORTED COFLEX IMPLANTATION. [22867 (CPT®)]				
Laterality	Wound Class	Incision Closure	Anesthesia	Op Region
N/A	Clean		General	

Procedure Info (continued)

Diagnosis (required)

Post-op diagnosis: Lumbar radiculopathy [M54.16]

Pre Vitals Current as XXXXXXXXXX

Current as XXXXXXXXXX

Pulse: 79

SpO2: 98

Weight: 68 kg (150 lb) (04/25/23)

edited 04/25/23 0836 by [REDACTED]

Problem List Current as

Current as

Lumbar stenosis with neurogenic claudication

Anesthesia History

No specialty history recorded

Other Medical History

Asthma

HTN (hypertension)

Surgical History

HX TUBAL LIGATION

PR COLONOSCOPY FLX DX W/COLLJ SPEC WHEN
PFRMD

Social History

Tobacco History

Smoking Status

Never

Smokeless Tobacco Use

Unknown

Alcohol History

Alcohol Use Status

No

Drug Use

Drug Use Status

No

All Component Based Labs (Last result in the past 60 days)

								Pregnan cy	
Hgb	Hct	WBC	Platelet	PT	INR	APTT	Preg Qual	Qualitati ve Urine	ABO/Rh
04/19/23	04/19/23	04/19/23	04/19/23			04/19/23			
1339	1339	1339	1339			1339			
13.6	42.1	5.41	254			35.2			

All Component Based Labs (Last result in the past 60 days)

[illegible]

All Component Based Labs (continued) (Last result in the past 60 days)

Sodium Lvl	Potassium Lvl	Chloride	CO2	BUN	Creatinine	Mean Plasma Glucose	AST	ALT
1339	1339	1339	1339	1339	1339	1339		
141	4.0	101	27	14	0.8	95		

All Component Based Labs (Last result in the past 60 days)

PT	INR	APTT	Preg Qual	Pregnancy Qualitative Urine
		04/19/23		
		1339		
		35.2		

All Component Based Labs (Last result in the past 60 days)

ABO/Rh	BUN	Creatinine	Mean Plasma Glucose
	04/19/23	04/19/23	04/19/23
	1339	1339	1339
	14	0.8	95

Pre Signoff**Prescription Medications**

Within last 14 days from 04/25/23

	Last Taken	Last Updated
Cholecalciferol (VITAMIN D3) 1000 UNIT CAPS	4/24/2023	04/25/23 0827
ciprofloxacin (CIPRO) 500 MG tablet	4/24/2023	04/25/23 0827
EPINEPHrine (EPIPEN) 0.3 MG/0.3ML DEVI auto-injector	Unknown	04/25/23 0827
levocetirizine (XYZAL) 5 MG tablet	Past Week	04/25/23 0827
Losartan Potassium-HCTZ (HYZAAR) 100-12.5 MG tablet	4/25/2023	04/25/23 0827
simvastatin (ZOCOR) 20 MG tablet	4/24/2023	04/25/23 0827
tiZANidine (ZANAFLEX) 4 MG tablet	Past Month	04/25/23 0827
HYDROcodone-acetaminophen (NORCO) 10-325 MG tablet	Past Week	04/25/23 0827

Preop Evaluation Note

Last edited 04/25/23 0916 by

Creation Time 04/25/23 0916

Status: Signed

Anesthesia Plan

ASA Score: 2

Mallampati: II

Possible difficult intubation? No

Anesthesia plan: General

Anesthetic plan and risks discussed with Patient.

Teeth Assessment: Normal

Facility Administered Medications

Facility Administered Medications (continued)

No medications found

Responsible Staff

04/25/23

Name	Role	Begin	End
[REDACTED]	ANESTH	1000	1202
	CRNA	1000	1202

Events

Date	Time	Event
4/25/2023	0916	Cleared for Anesthesia
	1000	Anesthesia Start
	1000	Start Data Collection
	1005	Induction
	1007	Intubation
	1030	Train of 4 Assess Train of 4 assessment is {Train of 4 Assessment:150111201} 0/4
	1030	Train of 4 Assess Train of 4 assessment is {Train of 4 Assessment:150111201}4/4
	1031	Quick Note INCISION.
	1136	Emergence
	1156	Extubation
	1158	Stop Data Collection
	1202	Anesthesia Stop

Block Note

No Anesthesia note exists

Device History

Device	Currently Attached?	Attached	Detached
MSC_OR_1 [170000001]	No	4/25/2023 10:00 AM START DATA COLLECTION	[REDACTED] STOP DATA COLLECTION

Checklist

	4/25/2023 0953
Room Temp Increased:	Yes
Re-evaluate Prior to Induct.:	Yes
Medcart Appropriately Stocked:	Yes
Sufficient Gas Supply:	Yes

Case Tracking Events

Event	Time In
In Procedure Room	1000
Procedure Started	1030
Procedure Completed	1150
Out of Procedure Room	1158

Lines, Drains, and Airways

Type	Details	Placement	Removal
Peripheral IV	04/25/23; 0846; Right; Wrist; 20 GA; 1; Tolerated Well; Yes; No; Patient discharged; 04/25/23;	[REDACTED]	[REDACTED]

Lines, Drains, and Airways (continued)

Type	Details	Placement	Removal
	1424		
ETT Group	04/25/23; 1007; Stylet, Mac #3; Straight ETT; Cuffed; 22; Oral; 7; Auscultation, EXCO2; 22 cm; 1; No; anterior larynx. cords not visualized, but + BBS. + EtCO2.; 04/25/23; 1156		

Positioning

	4/25/2023 1000	4/25/2023 1012	4/25/2023 1148
Position:	Supine	Prone	Supine
Field Avoidance:	N/A	N/A	—
Checklist:	PP Checked;Arms Tucked;Neck Stable	Eyes Taped;PP Checked;Neck Stable;Arms < 90;PP Padded	—
Comment:	—	placed prone using chest rolls x 2. prone foam headrest. eyes pressure free.	—

Attestation Information

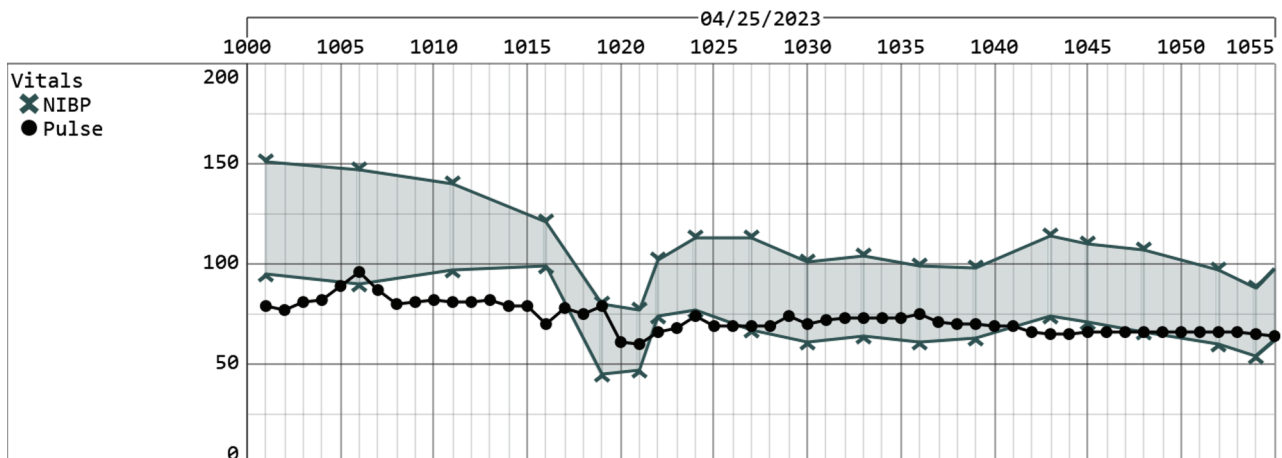
None

Assessments

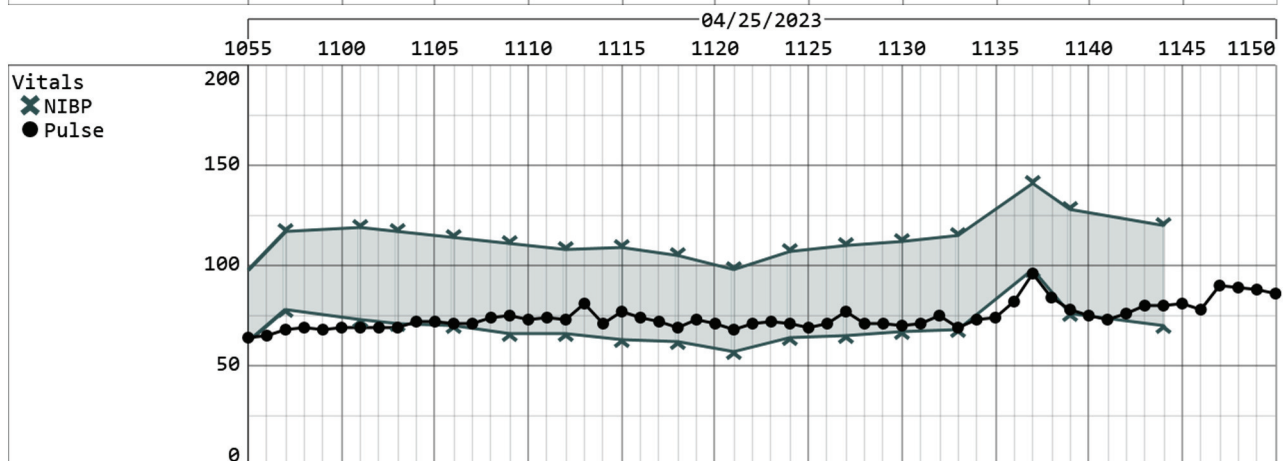
	4/25/2023 1001	4/25/2023 1016	4/25/2023 1149
L Breath Sounds:	Clear	Clear	Clear
R Breath Sounds:	Clear	Clear	Clear
Warming Device:	—	Intact	—
Eyes Free of Pressure:	Yes	Yes	Yes

Block Note

No Anesthesia note exists



O2 (L/min)	10											
O2 Inspired (%)	[97]	[94]	[92]	[93]	[94]	[94]	[94]	[94]	[93]	[93]	[93]	
O2 Expired (%)	[97]	[89]	[89]	[89]	[90]	[90]	[91]	[90]	[90]	[90]	[89]	
ETCO2 (mm/Hg)	[32]	[34]	[30]	[35]	[30]	[28]	[25]	[29]	[29]	[30]	[32]	
Sevo Inspired (%)		[2.9]	[2.3]	[3.4]	[2.4]	[2.3]	[2.3]	[2.4]	[2.4]	[2.5]	[2.6]	
Sevo Expired (%)		[1.9]	[1.8]	[2.2]	[2]	[1.9]	[1.9]	[1.9]	[1.9]	[2]	[2]	
dexamethasone... (mg)		4										
Fentanyl 100... (mcg)		50			50							
lidocaine (XY... (mg)		50										
propo... (mcg/kg/min)		150 mg			75							
Quelicin (suc... (mg)		100										
ondansetron (... (mg)						4						
ceFAZolin (KE... (mg)	2000											
ePHEDrine 50m... (mg)					15							
ketorolac (TO... (mg)									15			
lactated r... (mL/hr)	100											
ECG	NSR			NSR								
Vent Mode	S			SIMV V...								
Tidal Volume				460								
Resp	[12]	[12]	[12]	[12]	[12]	[12]	[12]	[9]	[8]	[8]	[6]	



[illegible]

PACU Handoff Notes

Last edited 04/25/23 1203 by [REDACTED]
Creation Time 04/25/23 1202
Status: Signed

Post Anesthesia Assessment

Patient: [REDACTED]

Procedure(s) Performed: Procedure(s):
L4-L5 LAMINECTOMY ABORTED COFLEX IMPLANTATION.

Post assessment: no apparent anesthetic complications, tolerated procedure well and no evidence of recall

Post vital signs: BP - BP: (!) 146/89, HR - 79, TEMP - 98.4 °F (36.9 °C), RR - 18, PULSE OX - 98 %

Level of consciousness: lethargic and responds to stimulation

Report given to RN

All Post Notes

Last edited 04/25/23 1448 by [REDACTED]
Creation Time 04/25/23 1441
Status: Signed

All Post Notes (continued)

Discharge Evaluation Note

Vitals in patient's normal range: **Yes**
Respiratory status is acceptable: **Yes**
Airway patency maintained: **Yes**
Cardiovascular status is stable: **Yes**
Hydration status is balanced: **Yes**
Patient participated in Post Anesthesia Evaluation: **Yes**
Pain is controlled: **Yes**
Nausea and vomiting controlled: **Yes**

Comments: **Patient woke up and complained of numbness and inability to move her right hand. After about 40 minutes she regained feeling in her hand but still had wrist drop. Puzzled as to why this happened. Pre op I asked specific questions about flexibility of neck and shoulders because she was going to be placed in the prone position. During the procedure she was carefully rolled over on the operating table on jelly chest rolls. Her shoulders were supported. Arms in neutral position. Head in foam head rest. Pulse ox. Was placed on her right hand with good reading. I consulted with Dr. Richter. Seeing that the feeling had returned to her hand he suggested waiting until tomorrow to see if this resolved. He would see her in the morning. The patient and her daughter agreed on this plan.**

Patient Demographics

Name	Patient ID	SSN	Gender Identity	Birth Date
Address	Phone	Email		
Reg Status	PCP	Date Last Verified	Next Review Date	

Subscriber Details

Hospital Account

CVG	Subscriber Name/Sex/Relation	Subscriber DOB	Subscriber Address/Phone	Subscriber Emp/Emp Phone

Guarantor Information

Name:		SSN:	
Address:			
City:		State:	
		Zip:	
Phone:			
Employer:			

Guarantor Information (continued)

Address: [REDACTED]
City: [REDACTED] **State:** [REDACTED] **Zip:** [REDACTED] **Phone:** [REDACTED]
Guar DOB: [REDACTED]

General Information

Date: 4/25/2023 Case classification: Scheduled Status: Unposted
Location: [REDACTED] Room: OR 03 Service: Neurosurgery
Patient class: Hospital Ambulatory
Surgery

Case Summary

Date: 04/25/23
Surgeon: [REDACTED]
Responsible: [REDACTED]
Location: M [REDACTED]
Procedure [Code]
L4-L5 LAMINECTOMY ABORTED COFLEX IMPLANTATION. [22867 CPT (R)]
Diagnosis [Codes]
Lumbar radiculopathy [M54.16]
Preoperative diagnosis
Lumbar radiculopathy [M54.16]

Responsible Staff

04/25/23

Name	Role	Begin	End
[REDACTED]	ANESTH	1000	1202
	CRNA	1000	1202

Preop Evaluation Note

Last edited 04/25/23 0916 by [REDACTED]
Creation Time 04/25/23 0916
Status: Signed

Anesthesia Plan

ASA Score: 2
Mallampati: II
Possible difficult intubation? No
Anesthesia plan: General
Anesthetic plan and risks discussed with Patient.
Teeth Assessment: Normal

Anesthesia Plan

No data found in the last 1 encounters.

Events

Date	Time	Event
4/25/2023	[REDACTED]	Cleared for Anesthesia
		Anesthesia Start
		Start Data Collection
		Induction
		Intubation
		Train of 4 Assess

Events (continued)

Date	Time	Event
	1030	Train of 4 Assess
	1031	Quick Note
	1136	Emergence
	1156	Extubation
	1158	Stop Data Collection
	1202	Anesthesia Stop

Case Tracking Events

Event	Time In
In Procedure Room	1000
Procedure Started	1030
Procedure Completed	1150
Out of Procedure Room	1158

Lines, Drains, and Airways

Type	Details	Placement	Removal
Peripheral IV	04/25/23; 0846; Right; Wrist; 20 GA; 1; Tolerated Well; Yes; No; Patient discharged; 04/25/23; 1424		
ETT Group	04/25/23; 1007; Stylet, Mac #3; Straight ETT; Cuffed; 22; Oral; 7; Auscultation, EXCO2; 22 cm; 1; No; anterior larynx. cords not visualized, but + BBS. + EtCO2.; 04/25/23; 1156		

Operative Reports**Brief Op Notes signed by** [REDACTED]

Author: [REDACTED] Service: Neurosurgery Author Type: Nurse Practitioner
Filed: 4/ [REDACTED]
Editor: [REDACTED]

BRIEF OPERATIVE NOTE

NAME: [REDACTED] MR#: [REDACTED]
[REDACTED]

SURGEON:
[REDACTED]

PREOPERATIVE DIAGNOSIS:
Lumbar radiculopathy [M54.16]

POSTOPERATIVE DIAGNOSIS:
Lumbar radiculopathy [M54.16]

OPERATION PERFORMED:
Procedure(s):
L4-L5 LAMINECTOMY ABORTED COFLEX IMPLANTATION. - Wound Class: Clean

ANESTHESIA:

Operative Reports [REDACTED]Brief Op Notes signed by [REDACTED]

General

FINDINGS:

Brittle bone

COMPLICATIONS:

None

ESTIMATED BLOOD LOSS:[REDACTED] ccElectronically Signed by [REDACTED]**Checklist**

	4/25/2023 0953
Room Temp Increased:	Yes
Re-evaluate Prior to Induct.:	Yes
Medcart Appropriately Stocked:	Yes
Sufficient Gas Supply:	Yes

Positioning

	4/25/2023 1000	4/25/2023 1012	4/25/2023 1148
Position:	Supine	Prone	Supine
Field Avoidance:	N/A	N/A	—
Checklist:	PP Checked;Arms Tucked;Neck Stable	Eyes Taped;PP Checked;Neck Stable;Arms < 90;PP Padded	—
Comment:	—	placed prone using chest rolls x 2. prone foam headrest. eyes pressure free.	—

Block Note

No Anesthesia note exists

Final Anesthesia Type

Type: General

PACU Handoff NotesLast edited 04/25/23 1203 by [REDACTED]

Creation Time 04/25/23 1202

[REDACTED]**Post Anesthesia Assessment**

PACU Handoff Notes

Patient:

Procedure(s) Performed: Procedure(s):

L4-L5 LAMINECTOMY ABORTED COFLEX IMPLANTATION.

Post assessment: no apparent anesthetic complications, tolerated procedure well and no evidence of recall

Post vital signs: BP - BP: (!) 146/89, HR - 79, TEMP - 98.4 °F (36.9 °C), RR - 18, PULSE OX - 98 %

Level of consciousness: lethargic and responds to stimulation

Report given to RN

All Postprocedure Notes

Last edited 04/25/23 1448 by

Creation Time 04/25/23 1441

Discharge Evaluation Note

Vitals in patient's normal range: **Yes**

Respiratory status is acceptable: **Yes**

Airway patency maintained: **Yes**

Cardiovascular status is stable: **Yes**

Hydration status is balanced: **Yes**

Patient participated in Post Anesthesia Evaluation: **Yes**

Pain is controlled: **Yes**

Nausea and vomiting controlled: **Yes**

Comments: Patient woke up and complained of numbness and inability to move her right hand. After about 40 minutes she regained feeling in her hand but still had wrist drop. Puzzled as to why this happened. Pre op I asked specific questions about flexibility of neck and shoulders because she was going to be placed in the prone position. During the procedure she was carefully rolled over on the operating table on jelly chest rolls. Her shoulders were supported. Arms in neutral position. Head in foam head rest. Pulse ox. Was placed on her right hand with good reading. I consulted with Dr. Richter. Seeing that the feeling had returned to her hand he suggested waiting until tomorrow to see if this resolved. He would see her in the morning. The patient and her daughter agreed on this plan.

Electronically signed by

Timeout Information

Timeout Type -: Preprocedure

Verification Staff

Performed Instant

4/25/2023 10:30 AM

Sup

Staff

Correct patient?: Yes

Correct site?: Yes

Correct position?: Yes

Timeout Information

Correct procedure?: Yes

Are the H&P and consent complete and verified, and in agreement with the scheduled procedure?: Yes
 Radiology studies available?: N/A
 Anesthesia states that monitoring is on patient?: Yes
 Monitoring equipment on patient?: Yes
 Irrigations / Medications available?: Yes
 Irrigations / Medications available?: Yes
 Confirmed correct radiology images for patient?: N/A
 Comments: BRETT SZYMANSKI COFLEX REP

Intra-procedure Meds**dexamethasone (DECADRON) 4 mg/mL injection (mg)**

Date/Time	Rate/Dose/Volume	Action
04/25/23 1006	4 mg	Given

Fentanyl 100MCG/2ML inj (mcg)

Date/Time	Rate/Dose/Volume	Action
04/25/23 1005	50 mcg	Given
1020	50 mcg	Given
1116	50 mcg	Given

lidocaine (XYLOCAINE) 2 % injection (mg)

Date/Time	Rate/Dose/Volume	Action
04/25/23 1005	50 mg	Given

propofol (DIPRIVAN) 200mg/20mL injection (mcg/kg/min) Dosing weight: 68

Date/Time	Rate/Dose/Volume	Action
04/25/23 1005	150 mg	Given
1020	■ mcg/kg/min - 30.6 mL/hr	New Bag
1202		Stopped

Comment: Anesthesia infusion automatically stopped by extension.

Quelicin (succinylcholine) 20 mg/mL injection (mg)

Date/Time	Rate/Dose/Volume	Action
04/25/23 1005	100 mg	Given

ondansetron (ZOFTRAN) 2 mg/mL injection (mg)

Date/Time	Rate/Dose/Volume	Action
04/25/23 1026	4 mg	Given

ceFAZolin (KEFZOL/ANCEF) 1 g IM/IVPush injection (mg)

Date/Time	Rate/Dose/Volume	Action
04/25/23 1000	2,000 mg	Given

ePHEDrine 50mg/ml injection (mg)

Date/Time	Rate/Dose/Volume	Action
04/25/23 1022	15 mg	Given
1057	10 mg	Given

ketorolac (TORADOL) injection (mg)

Date/Time	Rate/Dose/Volume	Action
04/25/23 1043	15 mg	Given

lactated ringers infusion (mL/hr) Dosing weight: 68

Date/Time	Rate/Dose/Volume	Action
04/25/23 1001	100 mL/hr	New Bag
1112	100 mL/hr - 700 mL	New Bag

MR Coding Query

Patient's encounter data is missing

MR Coding Query [REDACTED]

Final Anesthesia Type

Type: General

MRN: [REDACTED]

Richter, Erich O, MD
Physician
Neurosurgery

OR Surgeon 
Signed

Creation Time: 4/25/2023 12:37 PM

Case Time:
4/25/2023 9:30 AM

Procedures:
[REDACTED]
COFLEX IMPLANTATION.

Surgeons:
[REDACTED]
Pobanz, Rhonda L, NP

COVENANT HEALTHCARE
Saginaw, Michigan

OPERATIVE REPORT

NAME: [REDACTED]
[REDACTED] DOB: [REDACTED]
[REDACTED] CSN: [REDACTED]

Date of Service: 4/25/2023

Preop Diagnosis: Lumbar radiculopathy [M54.16]

Postop Diagnosis: Lumbar radiculopathy [M54.16]

Procedure(s):
L4-L5 LAMINECTOMY ABORTED COFLEX IMPLANTATION. - Wound Class: Clean

Surgeon: [REDACTED]

Assistant: Rhonda Pobanz, ARNP

Anesthesia Type: General

EBL: See Anesthesia Record

Fluids: See Anesthesia Record

Drains: None

Disposition: To PACU and then to

Implants: None

Operative Summary:

The patient was brought to the Major operating suite, and after induction of general anesthesia, was positioned prone on the operating table on a Wilson frame. Attention was paid to the padding of all pressure points, neuro monitoring was initiated, a time-out was performed and

preoperative antibiotics were verified. AP and lateral fluoroscopy were used to examine the lumbar spine and select an appropriate midline incision for approach to the L4-5 interspace.

The intended incision was injected with 0.25% Marcaine with epinephrine and incised sharply. Hemostasis was obtained with electrocautery and a self retaining retractor was placed. The dissection was carried down in the midline to the lumbodorsal fascia.

Having thus verified the levels for approach with bony landmarks on intraoperative imaging, the incision was extended as necessary and the periosteal dissection carried down along the spinous processes and lamina to the base of the facet complex bilaterally. The self adjusting retractors were adjusted to for this depth of the exposure, and the caudal portion of the superior spinous process was removed with a large bite Leksell rongeur. The Midas-Rex drill was used with a 3 mm match stick bit to divide the lamina from the facet complex down to the level of the ligamentum flavum bilaterally and to thin the lamina to the inner cortex to allow removal with a Kerrison rongeur. The ligament was then separated and elevated from the epidural fat and dura with an angled curette and areas of adhesion were separated with an angled curette and a straight curette. The ligamentum flavum and remaining lamina removed piecemeal with Kerrison rongeurs. Hemostasis was maintained with FloSeal, Gelfoam and bone wax, as well as bipolar electrocautery. The wound was copiously irrigated. I placed a laminar spreader because of the amount of disc space collapse and interspace stenosis but the bone was quite weak and sheared. No matter how deep I placed it, any direct traction on the bone led to bony disruption. Accordingly, I did not feel that the bone was strong enough to hold an implant and thought the risk of migration was quite high and aborted the Coflex implantation. We continued with laminectomy and decompression. The bony decompression was carried laterally to the lateral aspect of the spinal canal, and the shelving edge of the ligamentum flavum was removed with Kerrison rongeurs. Hemostasis was maintained with topical agents. The decompression was continued until this level had been well decompressed and the dura had returned to its normal shape. Medial facetectomy was performed bilaterally and the nerve hook was passed laterally to explore the decompression.

Attention was turned to wound closure. The fascia and subcuticular tissue was closed in layers with absorbable suture, and the skin was closed with skin glue. All counts were correct at the end of the case, the patient tolerated the procedure well and was transferred to PACU in good condition.

Rhonda Pobanz, ARNP assisted with positioning, opening, retraction, and closure.

Admission (Discharged) on 4/25/2023

Note shared with patient

Care Timeline

04/25 1000 L4-L5 LAMINECTOMY ABORTED COFLEX IMPLANTATION.
04/25 1450 Discharged 1450